DRUGS · PAIN AND NAUSEA

Drugs for neuropathic and facial pain

Neuropathic and facial pain present to ENT surgeons as ear, throat, tongue, face and sinus pain without a structural cause: trigeminal and glossopharyngeal neuralgia, burning mouth syndrome, post-herpetic neuralgia, persistent idiopathic facial pain, and neuropathic pain after neck dissection or other surgery. The drugs are anticonvulsants and antidepressants, used at doses built up slowly.

1 CHAPTER ONE At a glance

The drugs on one page.

1.1 Summary

DRUGS FOR NEUROPATHIC AND FACIAL PAIN

	SUMMARY
Carbamazepine	First-line for trigeminal and glossopharyngeal neuralgia: 100 mg once or twice daily, increased gradually to the lowest effective dose (commonly 200 mg three or four times daily, maximum 1.6 g a day); oxcarbazepine is an alternative
Gabapentin	300 mg at night, increasing by 300 mg every few days to 900 to 3,600 mg a day in three doses
Pregabalin	75 mg twice daily, increasing to 150 to 300 mg twice daily
Amitriptyline	10 mg at night, increasing to 25 to 75 mg (nortriptyline is better tolerated)
Duloxetine	60 mg once daily
Burning mouth syndrome	Topical clonazepam (dissolved in the mouth and spat out), alpha-lipoic acid, cognitive behavioural therapy, amitriptyline, gabapentin

	SUMMARY
ENT uses	Trigeminal and glossopharyngeal neuralgia; burning mouth syndrome; post-herpetic neuralgia (after Ramsay Hunt or trigeminal zoster); persistent idiopathic facial pain; post-surgical neuropathic pain; chronic tension-type headache presenting as "sinus" pain

2 CHAPTER TWO Using them in ENT

Practice.

2.1 Neuralgias

Trigeminal neuralgia causes electric-shock pains triggered by touch, eating or talking; **glossopharyngeal neuralgia** causes similar pains in the throat, tongue base and ear on swallowing, occasionally with syncope. Carbamazepine helps most; MRI looks for a vascular loop, multiple sclerosis or a tumour; microvascular decompression is considered when drugs fail. Eagle syndrome (elongated styloid) is a differential.

2.2 "Sinus" headache

Most patients referred with facial pain and a normal nasal endoscopy and CT do not have sinusitis; migraine, tension-type headache and midfacial segment pain respond to amitriptyline or migraine treatment, not to sinus surgery.

3 CHAPTER THREE Safety

What goes wrong.

3.1 Adverse effects

- **Carbamazepine**: dizziness, ataxia, diplopia, hyponatraemia, leucopenia, raised liver enzymes; **Stevens-Johnson syndrome**, strongly linked to HLA-B*15:02 in patients of Han Chinese, Thai and some other Asian ancestry (test before starting); potent enzyme inducer (reduces contraceptive pill, warfarin and many other drugs); teratogenic.
- **Gabapentinoids**: drowsiness, dizziness, weight gain, misuse and dependence (controlled drugs in the UK), respiratory depression with opioids.
- **Tricyclics**: anticholinergic effects, sedation, postural hypotension, cardiac toxicity in overdose.

4 CHAPTER FOUR For the examination

What the examiner wants to hear.

4.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- Features of **glossopharyngeal neuralgia** and its treatment.
- **HLA-B*15:02** testing before carbamazepine.
- Facial pain that is not sinusitis.

4.2 Common pitfalls

COMMON PITFALLS

- Sinus surgery for facial pain with a normal CT.

- Starting carbamazepine at full dose.

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